National Cancer Control Programme (NCCP) 00314 · clinician draft

Cisplatin + fluorouracil every 4 weeks

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Days 1–4 · multiday cisplatin / fluorouracil

Active medicines: Cisplatin + Fluorouracil

Emetogenic risk: high

Medicine	Dose / route	When
Multiday high-risk antiemetics	Prescriber/pharmacist to complete the four-day schedule and delayed cover	Before each treatment day and for two days after the final systemic anticancer therapy (SACT) day under NCIS multiday guidance Do not apply the single-day Day 1–3 aprepitant course as a complete four-day cisplatin prescription. The separate NCIS five-day cisplatin note does not directly specify this four-day regimen.
Cisplatin pre-hydration and electrolytes	Individual prescription required	Before cisplatin on Days 1–4 NCCP offers a suggested saline/magnesium regimen; volume and potassium need individual renal/cardiac and laboratory review.

Additional phase-specific support

- Prescribe regimen-specific hydration and electrolyte replacement with fluid-balance review. The appropriate volume depends on kidney and cardiac status; a generic printed plan is not an intravenous-fluid prescription.
- Review hearing, tinnitus and peripheral nerve symptoms before each cycle; damage can persist. Review nephrotoxic, ototoxic and other interacting medicines.
- Record the dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) result before the first fluorouracil exposure unless previously tested. Complete deficiency precludes fluorouracil; partial deficiency needs an individual starting dose and careful subsequent review.
- Normal DPD testing cannot exclude all severe fluorouracil toxicity. Explain unexpectedly early or severe diarrhoea, mucositis, neurological symptoms and chest pain.
- Write the actual infusion option, access arrangement, bag or pump changes and disconnect time. Do not equate four treatment days with a single universal pump duration.
- Provide an individually prescribed diarrhoea plan and hand/foot care advice; severe or persistent symptoms require assessment rather than repeated self-treatment.
- Arrange nutrition, weight and swallowing review early. Dietitian and speech and language therapy input should follow the person's needs; swallowing safety cannot be inferred from a general leaflet.
- Use the agreed mouth-care and dental plan. Report painful swallowing, mouth ulceration, weight loss or inability to drink before the next appointment.
- Reconcile prescription, over-the-counter and complementary medicines. Discuss fertility, contraception and breastfeeding using the actual products and individual circumstances.
- Confirm the hospital's urgent contact and fever instructions. Infection, bleeding, severe diarrhoea, dehydration, chest pain or new breathlessness need prompt assessment, not the next routine review.
- The regimen is high emetic risk on its four cisplatin days. Review rescue treatment and persistence of sickness between visits; the unfilled schedule is intentional until the prescriber confirms a complete multiday course.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00314 v4b · pNCCP 00314: treatment and supportive-care sections; NCIS v8 pp4–5 where used · NCCP 00314 and current NCIS/local supportive-care guidance

Sources checked 2026-11-10 · SACTCheck v0.84.47 · Regimen-level drafts; prescriber and oncology-pharmacy verification required